

2027 GSK DAC Creative Brief-Shingles & Meningitis

From Campaign-led To Patient-led Annual Engagement

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OUR AMBITION: BUILD RELEVANCE, NOT JUST CAMPAIGNS

TODAY

- **Strong communication foundation** A rich portfolio of KVs, videos, content assets, activations and engagement platforms is already in place.
- **Communication is still largely disease-led** Content often starts with disease messages or individual campaign topics.
- **Multiple campaigns and activations throughout the year**
- **Digital and social engagement is established, but largely remains online**

THE CHALLENGE

- **Rich resources, but campaign-by-campaign impact** Assets and activations are developed campaign by campaign, without an overarching annual framework to define their roles, connections and continuity, so the impact each one creates is not carried forward, connected or amplified over time.
- **Disease-first rather than audience-first** We are not yet fully leveraging what people care about—their concerns, interests, behaviors and life contexts—as natural entry points to the disease.
- **Engagement does not yet fully translate into advocacy and action** Limited organic conversation and sharing among consumers, while a gap remains between online engagement and real-world HCP consultation.

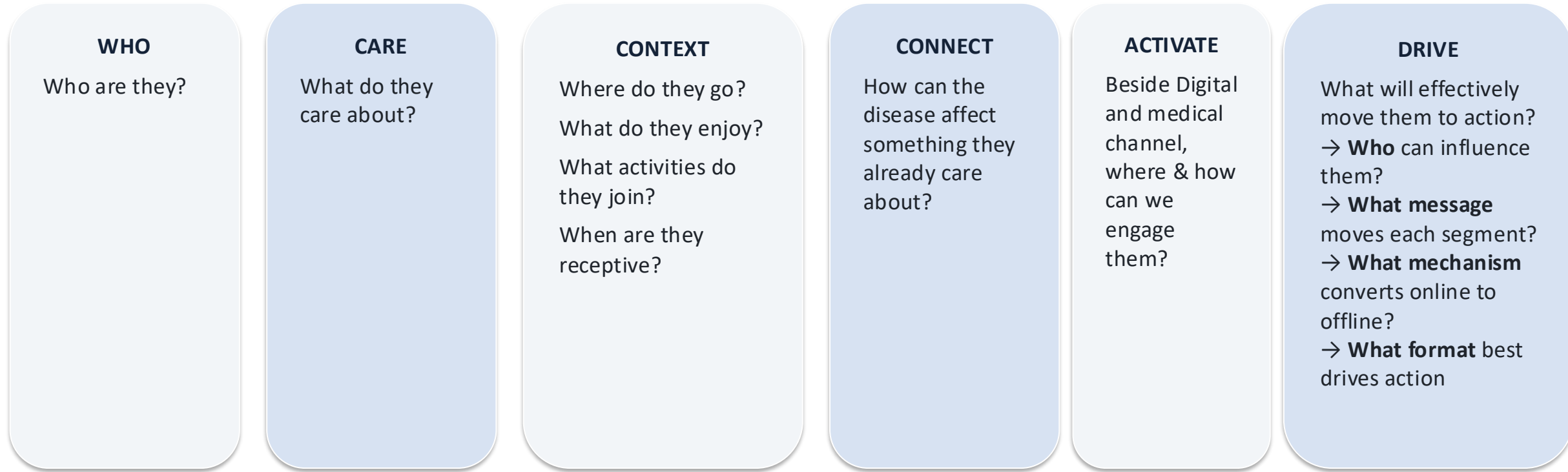
2027 AMBITION

FROM GREAT CAMPAIGNS TO ONE IMPACTFUL SYSTEM

- **ORCHESTRATED, CUMULATIVE ECOSYSTEM** Connect existing resources and future ideas under one annual strategy, with a clear purpose and role for every asset and activation, so each activation builds on the last and continuously strengthens relevance and impact over time.
- **PATIENT-LED** Start with what matters to people, then connect those concerns and life contexts to disease risk and severity to build stronger personal relevance.
- **ACTION-DRIVEN** Create ideas people want to participate in, talk about and share, while building a clear O2O journey that turns engagement into real-world consultation.

Move from what we want to say → to what already matters to people.

START WITH PEOPLE, NOT THE DISEASE



Creative ideas should be the output of audience understanding — not the starting point.

Your role is not to create the next campaign, but to orchestrate an annual creative ecosystem where every existing and future asset has a purpose, works together, and builds cumulative impact

DAC SUCCESS CASES: WHAT WE LEARNED

PAST EXAMPLE

Mazu Blessing Card

OPPORTUNITY

Praying for blessings and safety is already something people care about and act on - an existing occasion and motivation.

WHAT WE DID

We built disease education into the temple experience and the blessing card, creating a natural reason to visit a clinic for a consultation and gain complete protection.

LEARNING

Starting from what people already care about creates a meaningful consultation trigger. → links to 2027 relevance & consultation enablers.

PAST EXAMPLE

LINE Short Drama: Unlock the Ending at the Clinic

OPPORTUNITY

Story suspense makes people want to know what happens next.

WHAT WE DID

We made the drama's ending unlockable only at the clinic, extending online engagement offline.

LEARNING

O2O is not just traffic - it is giving people a reason worth taking the next step for. → links to 2027 consultation enablers.

PAST EXAMPLE

Kindergarten Health Education Kits

OPPORTUNITY

Kindergartens are an everyday setting where parents naturally receive health information.

WHAT WE DID

Colouring and puzzle materials kids love created new reach, linked to nearby clinics to take consultations.

LEARNING

Going deeper into the target audience's daily life uncovers new reach and consultation opportunities. → links to 2027 physical reach & experience.

Past examples are a springboard, not a template.

4 CHALLENGES SHARED BY BOTH DISEASES

The three challenges on the previous page, translated into four execution-level tasks — the solutions do not have to be the same. The following defines the expected outcome, not the creative format.

01

ATTENTION

How can we surface new topics and formats people genuinely care about — without reshooting the hero film?

02

PARTICIPATION

How can we create online activities people want to join, discuss and share?

03

PHYSICAL REACH

How can we create memorable, influential physical contact rather than one-way health education?

04

ACTION

How can we convert digital attention and intent into a simple, incentivized next step that clinics can receive?

Creative freedom: content platforms, partnerships, community actions, experiences, services, missions, tools — or an unexpected combination.

SHINGLES | THE CHALLENGE WE NEED TO SOLVE

WHAT WE HAVE

Established creative platform "Shingles doesn't play fair — and it's never just pain"

Audience-led hero content CVRM & Caregiver content built around audience-preferred formats and interests

- Hypertension / CVD: Palace drama
- Diabetes: Sports competition
- Kidney disease: TV talk show
- Flu season: K-drama / CEO romance
- Caregivers: "Shingles bully" threatening parents to trigger children's protective instinct and action

Current content primarily builds risk information, while severity and broader consequences are introduced at a lighter level.

Engagement ecosystem Website • LINE OA • HCP consultation Map • PR / Earned Media

O2O consultation mechanisms

- Offline → Ongoing Engagement: Physical activations drive LINE OA acquisition for continued engagement.
- Cultural Activation → Consultation: Temple activations connect culturally relevant experiences with HCP consultation.
- POC Engagement: Interactive POSM increases attention and engagement with disease information in clinics.
- Online → Offline: Digital short-drama activation encourages clinic visits through an "unlock the ending at clinic" mechanism.

SHINGLES | 2026 STATUS, INSIGHTS & OPPORTUNITIES

	AWARENESS	CONSIDERATION	CONSULTATION — INTENT & ACTION
KEY STATUS	Severity T2B TTL 55.0% → 55.4% Stable CV-risk recognition CVRM 38.0% → 43.4% +5.4 ppt Message exposure TTL Pain 63.3% Complications 32.9%	Information seeking TTL 37.0% → 45.4% +8.4 ppt Worry T2B TTL 30.0% → 35.5% +5.5 ppt CVRM tension Severity tied to complications 53.8% Reassured by good control 41.8% Caregiver-driven consultation 44.6% Trusted voices: HCP 45.5% Patient stories 34.0% Celebrity 20.9%	Proactive consultation TTL 19.0% → 22.0% +3.0 ppt Consultation intent TTL 19.0% → 22.6% +3.6 ppt Journey CVRM Before consultation needed 4.27 exposures 6.11 months to HCP inquiry
KEY INSIGHT	Pain remains established, while higher CV-risk recognition shows growing understanding beyond pain.	Interest and concern are rising, but good chronic-disease control can still make shingles feel personally distant.	Consultation momentum is building, but the path from intent to conversation remains long and multi-touch.
KEY ACTION	01 Make Beyond Pain Personally Relevant Connect growing understanding to the health priorities CVRM patients already work to protect.	02 Counter False Reassurance Make the risk credible and harder to dismiss despite well-controlled chronic disease.	03 Turn Intent Into HCP Conversation Create a clearer path from intention to active HCP inquiry.

T2B: Top 2 Box; TTL: Total sample; CV: cardiovascular; CVRM: Cardiovascular, Renal, and Metabolic subgroup; ppt: percentage points.
Source: Ipsos shingles & RSV HCC ATU Market Research, Q3 2026.

SHINGLES | 2027 DAC STRATEGY

From Severity Awareness to Personally Relevant HCP Conversation

CORE CHALLENGE

Awareness is established and broadening beyond pain, but stronger personal relevance and sustained engagement are still needed to accelerate HCP conversation.

2027 FOCUS

Make shingles personally relevant to CVRM patients and convert rising intent into HCP conversation. Build relevance, reinforce credible prompting and connect the path to conversation.

01 Build Personal Relevance

From “I understand the risk” to “this matters to my health.”

Connect the potential impact of shingles to the health priorities CVRM patients already work to protect.

- Chronic-disease health priorities
- Personally relevant consequences

02 Activate Trusted Voices

From “my condition is controlled” to “this risk still deserves attention.”

Use HCP credibility and patient/family prompting to counter false reassurance and strengthen consideration.

- HCP and healthcare-linked voices
- Patient-peer and family prompting

03 Enable HCP Conversation

From “I intend to ask” to “I start the conversation.”

Connect repeated education, healthcare-linked touchpoints and clear cues to help turn intent into HCP inquiry.

- Repeated community education
- Connected O2O journey
- POC conversation cues

SHINGLES | 2027 CREATIVE ASSIGNMENT

Expected outputs — all connected under one annual relevance approach

01 RELEVANCE IDEA

ONE idea under the existing "Shingles doesn't play fair" platform

- Start from what people care about, experience & believe
- Connect risk + severity to real-life concerns
- Flex across CVRM, Caregiver & relevant moments
- Raise relevance across the year — not reinvent the platform or run multiple standalone campaigns

02 CONTENT & TRUST

Maximize what we have; fill the gaps — no new hero OTV

- Keep / Reframe / Repurpose / Create existing assets
- Create new formats across relevant touchpoints — not just adaptations
- Reimagine HCP / real-patient / celebrity-patient voices
- Beyond traditional videos & banners: interaction-first, the "yes, that's exactly what I was thinking" reaction

03 EXPERIENCE & SOCIAL

Create experiences people want to join, talk about & share

- ONE scalable, repeatable immersive Shingles experience
- Integrate Shingles experience + Patient voice + HCP education
- Digital buzz / social conversation + dedicated Caregiver campaign
- Repeatable at small scale across multiple occasions; not limited to traditional health-education events

04 CONSULTATION ENABLERS

Design stronger reasons to move from engagement to consultation

- Optimize O2O conversion mechanisms
- Go beyond traffic-driving / QR-code mechanics
- Create POC contextual conversation enablers
- Find the real trigger or reason that makes people take the next step and raise it naturally with an HCP

MANDATORY | ANNUAL ORCHESTRATION

Show how ALL existing + new assets work together across the year — not as a tactics shopping list. The same orchestration requirement applies to the Meningitis assignment.

WHO it is for → WHAT insight → ROLE in journey → WHAT action it drives

RELEVANCE → ENGAGEMENT → TRUST / CONSIDERATION → HCP CONSULTATION

Build on the existing platform. Connect every output into ONE annual plan that grows relevance and moves people toward consultation.

Every proposal must answer clearly: who it is for, why it resonates, how it builds trust, which audience state it changes, how it moves them forward, and what finally makes them consult an HCP.

MENINGITIS | THE CHALLENGE WE NEED TO SOLVE

WHAT WE HAVE

Established creative platform “The scariest thing about meningitis is not being afraid of it”

Audience-led hero content Anchored in each parent segment’s concern for their child, using relatable everyday scenarios to make meningitis risk visible and overturn the “it won’t happen to my child” myth

- Infant: 50% mortality and the unpredictability of bacterial transmission
- Toddler: deterioration within a single day; early symptoms are hard to recognise
- Adolescent’s parents & adult: highest carriage rate; sharing and group living increase transmission

Current content starts mainly from disease severity, with less entry from parents’ everyday contexts and parenting concerns, and life-stage touchpoints are not yet connected into continuous year-round communication.

Engagement resources

- Website • LINE OA
- Postpartum care centre health education placement
- Kindergarten health education material distribution programme
- Toddler talent-development test machine | parent-community amplification campaign site

MENINGITIS | 2026 LEARNINGS & NEXT STEPS: FROM AWARENESS TO CONSULTATION

AWARENESS 71%

(+7ppt)

Has heard of IMD

UNDERSTANDING 22% (+6ppt)

Familiar with IMD

RELEVANCE 44%*

(-)

Sees risk for own child

CONSULTATION 24% (+16ppt)

Proactively consulting HCPs

KEY BARRIERS

Awareness exists, but touchpoints fade

Awareness is broadly consistent across life stages, but Toddler and AYA audiences have fewer HCP-led moments to make IMD personally relevant.

Awareness level: Infant 70%, Toddler 73%, AYA 70%

Severe, but "not my child"

Parents accept that IMD is serious, yet assume it will not happen to them, so urgency stays low.

Relevance level: Infant 55%, Toddler 43%, AYA 25%

Knowledge does not convert

Parents seldom raise IMD themselves, so awareness rarely turns into consideration or an actual consultation.

Proactive consultation: Infant 32%, Toddler 19%, AYA 20%

JOBS TO BE DONE

Create qualified education moments

Broader social platforms plus offline / EV71 (enterovirus 71) and flu moments, weighted to Toddler and A&A.

Make risk personal

Tie IMD to real-life settings and life-stage moments parents already recognize.

Drive consultation

Repeated HCP-led education and one clear next step for parents to take: Ask an HCP how to protect my child.

*Overall relevance is calculated based on the Q6.2 T2B perceived likelihood for each respondent's applicable life stage; partial N=169.

** AYA, Adolescent and Young Adults. A&A, Adolescents and Adults. IMD, invasive meningococcal disease. ppt: percentage points.

Source: Ipsos, IMD HCC Tracking Market Research 2026 Wave 1. Fieldwork: 21–26 Aug 2026; partial topline N=169. Percentages rounded; bases vary by question.

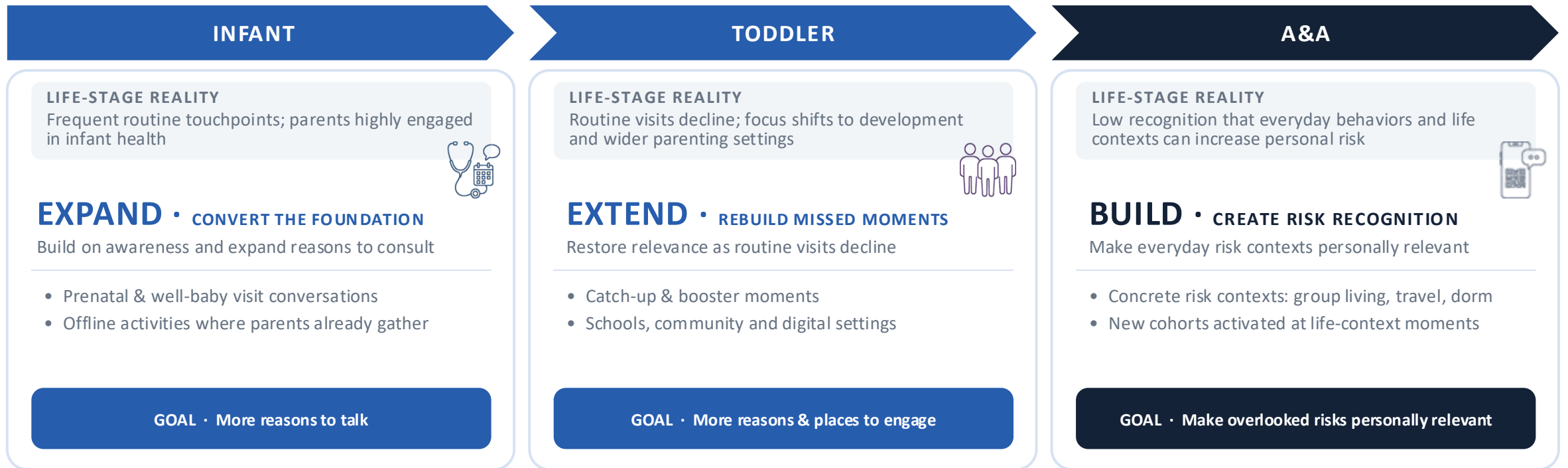
DAC is for disease awareness only and is not associated with brand promotion

Make Meningitis Personally Relevant at Every Life Stage

Move audiences from knowing meningitis is serious to feeling personally at risk and proactively consulting HCPs

2027 Theme: “The Scariest Thing About Meningitis is Not Being Afraid of It”

STRATEGIC DRIVERS BY LIFE STAGE



*A&A, Adolescents and Adults. IMD, invasive meningococcal disease. HCP, healthcare professional

2026 market research evidence is based on the study-abroad AYA segment; broader A&A cohorts and life-context opportunities require further validation.

DAC is for disease awareness only and is not associated with brand promotion

MENINGITIS | What We Want You to Create

Creative proposal grounded in life-stage relevance — not disconnected tactics

01 LIFE-MOMENT RELEVANCE

Make Meningitis relevant through real-life moments.

- Infant / Toddler / A&A: new lifestyle, parenting & life-moment topics
- A&A targeted digital campaign around relevant contexts*
 - Military / study abroad / dormitory / overseas work & travel / international school & camp

→ 從真實生活情境增加 Awareness & Relevance, 而非只講疾病

02 NEW REACH & EXPERIENCE

Find creative ways to enter young families' lives.

- Creative KOL collaboration
 - Children/family creators, educators or IPs — beyond conventional influencer endorsement
 - Creative family activation
 - Family interaction + education + Global assets + relevant life/risk contexts
 - Engaging, scalable & repeatable
- 深入家庭生活找到新的 Reach opportunity, 而非只增加媒體渠道

03 CONTENT, HCP & SOCIAL

Extend existing assets into engaging new formats.

- NEW lifestyle / parenting / life-moment content for ALL 3 TAs
- Reimagine HCP content beyond conventional educational videos
- Create digital/social ideas that drive conversation & organic buzz
- Not simply hero-film cut-downs or banners

→ 結合 TA 在意的話題, 創造更有互動、信任與討論性的內容

04 DRIVE CONSULTATION

Create stronger reasons to move from engagement to HCP consultation.

- Optimize O2O conversion mechanisms
 - Explore POC conversation enablers
 - Create a compelling reason / trigger to take the next step
 - Connect Digital / KOL / Activation / POC into a clear path
- O2O 不只是導流, 而是創造真正讓家長／消費者願意去問的理由與HCP自然可以開口的情境

CONNECT THE JOURNEY

LIFE MOMENT → RELEVANCE → ENGAGEMENT / TRUST → HCP CONSULTATION

Connect all deliverables into one coherent journey — not standalone tactics.

TIMELINE, BUDGET

OVERALL : THINK BEYOND INDIVIDUAL ASSETS OR EVENTS.

WE ARE LOOKING FOR A STRONG, SCALABLE CREATIVE IDEA THAT CAN EXTEND FROM CONTENT → DIGITAL/POSM → REAL-WORLD EXPERIENCE.

TIMELINE :

- **Before 9/24:** Deliver Big Idea proposal slide (Including a high-level budget proposal)
- **Late September / early October:** in-person presentation; date to be confirmed

BUDGET

*A&A, Adolescents' parent and Adults

SHINGLES

Asset | NT\$9M

Incl. AC & production for digital / print / online assets; excl. HCP, celebrity & patient advocate endorsement fees.

- Patient Stories: 3 themes × 3 episodes; short-form mini-series + digital adaptations
- HCP Content: 7 × 30-sec videos (HCP appearance can be 5–10 sec) + digital adaptations
- CVRM Scenarios: 3–6+ relevant topics; digital & POSM assets, preferably adapted from hero campaign content

Expectation: One scalable creative platform connecting all content, rather than standalone executions

Immersive Education Events | NT\$8M

- Approx. 50 events × 20 participants
- Scalable, immersive and interactive patient education experience
- Patient story-sharing is mandatory, but physical patient attendance is not required
- Concept should be easy to replicate across different locations

MDAC

Asset | NT\$3M

Incl. AC & production for digital / print / online assets; excl. HCP, celebrity & patient advocate endorsement fees.

- Infant/Toddler/A&A Scenarios: 10+ consumer-relevant lifestyle topics and content concepts digital (video+banner) & POSM assets, preferably adapted from hero campaign content
- A&A Online Campaign

Immersive Family Education Events | NT\$3.5M

- Approx. 20 participants/session
- Create an engaging parent–child experience that makes meningitis personally relevant
- Combine education with interaction / play rather than traditional health education
- Preferred: A scalable concept that can later be adapted and deployed directly into kindergartens / preschools

DAC GUARDRAILS

DAC / COMPLIANCE

- ✓ Educate disease risk, burden and action
- ✓ Drive discussion and consultation
- ✗ Product promotion
- ✗ Brand preference building
- ✗ Treatment superiority claims
- ✗ Brand or Company Color or Logo
- Avoid excessive fear appeals, inciting anxiety or simply amplifying disease severity; keep the focus on relevance and actionability
- Do not use AI to generate unapproved medical claims, fabricated patient testimonials or misleading HCP portrayals
- Any consultation activity may only be supported by an HCP
- HCP fees may only be paid by GSK